SAMU - Rwanda Biomedical Centre

SAMU Roster

User manual

For the PHTL

About this manual

This manual explains what the PHTL does in SAMU Roster: building the medical crew part of the roster, reviewing it and handling crew changes and shortages.

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Version 1 - 24 October 2026 - This manual covers only what this role does.

Your role

As **PHTL** you build and look after the **medical crew** part of the roster, including the medical staff who help cover the Regulation desk. Drivers and the dispatch desk are not touched by what you do.

You can	You cannot
Generate and regenerate medical crews	Generate drivers or the dispatch desk
Replace crew members; make a Pro the crew leader	Add or edit employees, stations, ambulances or leave
Accept a crew running short or without a Pro (with reason)	Publish a roster (administrator)
Check conflicts and submit a roster for review	Create events
Plan medical coverage for events; review command decisions	

Your menu

Dashboard, Generate Roster, Roster Board, Crew Rollup, Events, Notifications and Command Decisions.

Signing in and getting around

1. Open **https://ems-rw.duckdns.org** in a browser on a computer or phone.
2. Type your **username** and **password** exactly as given to you. The username is case sensitive and can contain a space.
3. Press **Sign in**.

> **IMPORTANT** After **5 wrong passwords** for the same username, sign-in is blocked for 15 minutes. You stay signed in for 12 hours, then you sign in again. There is no way to change your own password in the app: ask the administrator to set a new one.

On a phone

- Tap the **menu button** (three lines, top right). Groups with an arrow open when you tap them; tap a page and the menu closes.
- Lists appear as cards, one per person or record, with their buttons underneath. The roster grid scrolls sideways: swipe it.

Signing out and this manual

Log out and **User manual** are at the bottom of the menu, below your name. The manual you open is always the one for your role.

Generating the medical roster

1. Open **Generate Roster**. The page says *Generates medical crew only - drivers and dispatch aren't touched.*
2. Read the **Preflight check**, choose the **Range** and **Reference date**, and press **Generate roster**.
3. Wait. A full medical roster is the longest part and can take 10 to 15 minutes. You can keep the page open or return later.
4. The result shows the number of assignments, a quality score and any crew that is short.

- **Pro coverage:** every crew should have at least one Pro. Crew leaders can only be Pro staff.
- **Specialists:** anesthesiologists and critical-care staff work only at the four middle-site stations. They are also the only medical staff who can fill the Regulation desk beyond the call centre's own people, with a weekly limit per person.
- **Home station:** staff stay at their home station unless marked *Floats freely*, and are sent elsewhere only when another station is short.
- **Fairness:** total, night and weekend hours are balanced across the whole month. A day shift counts as 10 hours and a night shift as 14.
- **Regenerate** on the Roster Board recomputes only the medical crew and leaves drivers and dispatch alone.

How a roster moves from draft to published

Stage	What it means	Who acts
Draft	Being built. The three parts (drivers, medical crew, dispatch desk) can each be generated into the same draft.	You, the other managers and the administrator
Submitted	Sent for review. It is locked: nobody can generate into it.	Anyone signed in (you can submit when your part is done)
Published	Final. Every person in it is notified. Later changes go through crew replacements, which notify the people affected.	Administrator only

Check conflicts on the Roster Board lists anything that would block publishing: a missing rest day, a person scheduled while inactive, a night shift for someone not cleared for nights, a leave day that was overridden, a crew with no Pro, or an ambulance that is not in service. Each line says how to fix it.

Reviewing your part

1. Open **Roster Board** and choose the roster. You see medical staff only.
2. Press **Check conflicts**. A crew with no Pro, a missing rest day or a night shift for someone not cleared for nights blocks publishing until fixed or formally accepted.
3. When your part is ready, press **Submit for review**.

Crew leader and pulling people for events

- Click a Pro member's name in Crew Rollup to make them **crew leader** (only Pro staff can lead).
- The calendar-style button next to a crew member is **Pull for event**: it takes them off their crew and leaves it short, with no replacement.
- A replacement who is not a Pro, and who would leave the crew without one, needs an override.

Replacing someone or accepting a gap: Handle shortage

1. Open **Crew Operations > Crew Rollup**, choose the roster and the day, then pick the station. Each ambulance shift is a card.
2. Press the **swap button** next to the crew member (hover text: *Handle shortage*). A window opens: **Choose how to fill this seat**.
3. Under **Legal picks**, choose a person. Candidates are ranked by fairness, least-loaded first. **Home station only** and **Show ineligible too** filter the list. Press **Confirm crew replacement**. Both people are notified if the roster is already published.
4. If nobody is legal, **Risky picks - needs an override** lists people who would break a protected rule (for example a rest day or Pro coverage). Choose a reason, write a justification and type **OVERRIDE** to confirm.
5. If nobody can be found, under **Or: leave this crew short** press **Approve this crew to run short**. Choose a reason and write a justification. Nobody is moved; the gap is accepted and logged.

> **IMPORTANT** Every risky pick and every accepted shortage is recorded under **Command Decisions** with your name, the reason and the justification. Safety-related ones are reviewed by the administrator.

Planning event coverage

1. Open **Events** and find the event. If no generated roster covers the event date, you are told to plan a roster for that week first.
2. Press **Plan coverage** and choose **Roster to plan against**. The system computes a **Coverage plan**: ambulances, medical crew, and any gaps (*Gaps remain* or *Fully covered*).
3. If the plan would disrupt existing coverage (pulling an ambulance, reassigning someone from a shift or from mandated rest) it is marked **Risky**. Press **Continue - needs a reason**, choose a reason, write a justification, then **Confirm & commit**.
4. A plan with no disruption is committed with **Commit this plan**.

Command decisions and notices

- **Command Decisions** lists every approved rule break with who approved it and why. **Standby / on-call** people appear in Crew Rollup.

- **Notifications** shows messages for you. Press **Mark read**.

Reading the roster board

The board has one row per person and one column per day. Each cell holds a code. Colours: blue-tinted cells are day shifts, dark cells are night shifts, grey cells are days off, amber cells are leave or other absence, green cells are event duty.

Code	Meaning
D + number (D3)	Day shift at the station with that code
N + number (N3)	Night shift at the station with that code
DR / NR	Day / night shift at the Regulation desk (call centre desk)
DE / NE	Day / night event duty
O	Off
AL, SL, ML, PL	Annual leave, sick leave, maternity leave, paternity leave
TR, AD, P	Training, administration, permission
MS, ST, CY, CL	Mission, stock, cyclism, circumstance leave
DT, DM, DC, R	Data, day meeting, day checking, roster
SB	Standby

For fairness, a day shift counts as **10 hours** and a night shift as **14 hours**. The total on the right of each row is hours worked in the roster.

Rules the system applies when it builds a roster

- One shift per person per day. Leave is respected exactly as entered.
- The day after a night shift is a day off. A day shift is never placed directly before a night shift.
- No more than **2 working days in a row**. Rest and run-length rules also continue across the border between two rosters.
- People marked as unable to work nights never get night shifts.
- Anesthesiologists and critical-care staff work only at the four middle-site stations and are the only people the Regulation desk draws from the medical team.
- Every crew should have at least one Pro. If people are short, the system fills what it can and reports the gap; it never refuses to build a roster.
- Hours are balanced fairly across the whole calendar month, not only inside one roster.

If something goes wrong

What you see	What to do
Sign-in is refused	Check the username (capital letters, spaces) and password. After 5 wrong tries, wait 15 minutes.
'Session expired - please log in again'	You were signed in for more than 12 hours. Sign in again.
'Waiting for another generation to finish'	Only one roster is built at a time. Keep the page open or come back later.
Generation seems to take very long	A full roster takes about 10 to 15 minutes. If the page was closed or the system restarted, start it again.
'Would leave this crew with no Pro-rated member'	Pick a Pro as the replacement, or use an override with a written justification.
A crew is short	Replace someone, or approve the crew to run short (reason and justification required).
'Can't be regenerated (status submitted/published)'	The roster is locked. Ask the administrator to reopen a submitted one.
Anything else	Tell the administrator the screen name, what you pressed and the exact message.

Who to contact

	Name and phone (fill in)
Administrator	
Your own manager	